



Syncope

History

- Cardiac history, stroke, seizure
- Occult blood loss (GI, ectopic)
- Females: LMP, vaginal bleeding
- Fluid loss: nausea, vomiting, diarrhea
- Past medical history
- Medications

Signs and Symptoms

- Loss of consciousness with recovery
- Lightheadedness, dizziness
- Palpitations, slow or rapid pulse
- Pulse irregularity
- Decreased blood pressure

Differential

- Vasovagal
- Orthostatic hypotension
- Cardiac syncope
- Micturition / Defecation syncope
- Psychiatric
- Stroke
- Hypoglycemia
- Seizure
- Shock (see Shock Protocol)
- Toxicological (Alcohol)
- Medication effect (hypertension)
- PE
- AAA

Age Appropriate Airway Protocol(s) AR 1, 2, 3, 5, 6 <i>if indicated</i>	
	Blood Glucose Analysis Procedure
B	12 Lead ECG Procedure
	IV or IO Access Protocol UP 6
P	Cardiac Monitor
	Altered Mental Status Protocol UP 4 <i>if indicated</i>
	Age Appropriate Cardiac Protocol(s) <i>if indicated</i>
	Age Appropriate Hypotension/ Shock Protocol AM 5/ PM 3 <i>if indicated</i>
	Multiple Trauma Protocol TB 6 Spinal Motion Restriction Procedure/ Protocol TB 8 <i>if indicated</i>

Age Specific Blood Pressure indicating possible shock

Age 0 – 28 days: SBP < 60

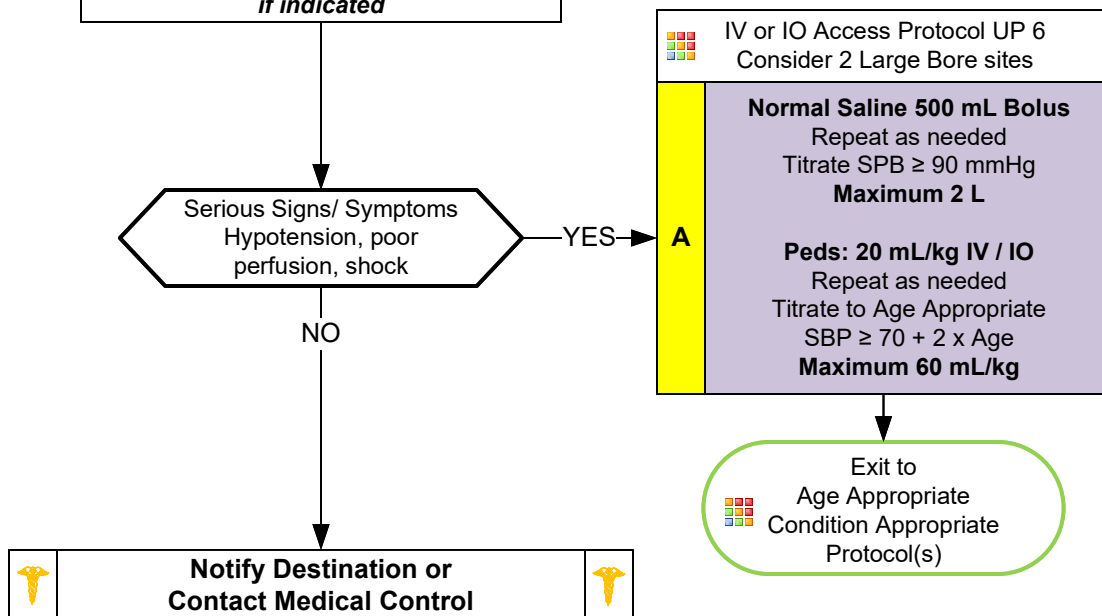
Ages ≥ 1 month: SBP < 70

Age 1 – 9: SBP < 70 + (2x Age)

Ages 10 – 64: SBP < 90

Ages ≥ 65: SBP < 110

All ages Shock Index:
HR > SBP





Syncope

Pearls

- **Recommended Exam: Mental Status, Skin, HEENT, Heart, Lungs, Abdomen, Back, Extremities, Neuro**
- **Syncope is both loss of consciousness and loss of postural/ muscle tone with collapse. Symptoms preceding the event are important in determining etiology.**
- **Syncope often is due to a benign process but can be an indication of serious underlying disease in both the adult and pediatric patient.**
- **Often patients with syncope are found normal on EMS evaluation. In general patients experiencing syncope require cardiac monitoring and emergency department evaluation.**
- **Differential should remain wide and include:**

Cardiac arrhythmia	Neurological problem	Choking	Pulmonary embolism
Hemorrhage	Stroke	Respiratory	Hypo or Hyperglycemia
GI Hemorrhage	Seizure	Sepsis	
- **High-risk patients:**

Age ≥ 60	Syncope with exertion
History of CHF	Syncope with chest pain
Abnormal ECG	Syncope with dyspnea
- **Abdominal/ back pain in women of childbearing age should be treated as pregnancy related until proven otherwise.**
- **The diagnosis of abdominal aneurysm should be considered with abdominal pain, with or without back and/ or lower extremity pain or diminished pulses, especially in patients over 50 and/ or patients with shock/ poor perfusion. Notify receiving facility early with suspected abdominal aneurysm.**
- **Consider cardiac etiology in patients > 35, diabetics, and/ or women especially with upper abdominal complaints.**
- **Heart Rate: Tachycardia is one of the first clinical signs of dehydration, typically increases as dehydration becomes more severe.**
- **Syncope with no preceding symptoms or event may be associated with an arrhythmia.**
- **Assess for signs and symptoms of trauma if associated or questionable fall with syncope.**
- **Consider dysrhythmias, GI bleed, ectopic pregnancy, and seizure as possible causes of syncope.**
- **In general these patients should be transported: Patients who experience syncope associated with headache, neck pain, chest pain, abdominal pain, back pain, dyspnea, or dyspnea on exertion need prompt medical evaluation.**
- **More than 25% of geriatric syncope is cardiac dysrhythmia based.**